



Occupational Therapy Pressure Injury Prevention and Management Procedure

1. Scope

Site	Operational Area	Applicable to
Armada Kalamunda Group	All services	Occupational Therapy

2. Purpose

Patients at risk of/or with pressure injuries will receive optimal Occupational Therapy assessment, intervention, and evaluation to prevent and assist in the care and healing of pressure injuries. Services will be in line with the [AKG Pressure Injury Prevention and Management Procedure](#)

3. Procedure

- Eligibility for Occupational Therapy pressure care services is as per general Occupational Therapy inpatient and outpatient service access criteria.

Inpatients with a current pressure injury or assessed by nursing staff as high risk will be assessed on the day of admission or the next day if admitted outside of Occupational Therapy service hours.

Other inpatients requiring pressure care intervention will be seen within two days.

- Outpatient referrals for clients with grade 2 or above pressure injuries or at high risk of pressure injuries are prioritised as priority 1 (Urgent) referrals and offered the earliest possible appointment.
- Outpatients transitioning from the ward are generally allocated priority 3 as they would have been set-up safely from the ward and are therefore not be at risk. Most patients are likely to be referred for follow-up or assessment for need for long term issue of items.
- Comprehensive assessment**
- A comprehensive pressure injury assessment is required for all patients referred or identified as requiring:
 - pressure injury care/risk assessment,
 - wheelchair provision, or
 - mattress replacement/overlays,
 - chair bound or bed bound patients
- Assessment items do not need completing if already conducted by another team member and is still relevant and available.

Before referencing this procedure document please ensure that you have the latest version from the [Policy Library](#) information hub page.

- Clinical history - including psychosocial issues that may impact on pressure injury prevention and management.
- Mobility and activity assessment
- Continence screen
- Cognitive screen – If functional difficulties complete screen – mini-cog, RUDAS, MoCA, Cognisant, MMSE as indicated.
- Assessment of extrinsic risk factors (shear, friction and moisture).
- Skin integrity risk assessment.
 - Braden Pressure Ulcer Risk Assessment (Norton or Waterlow can be used as an adjunct if desired)
 - Nutritional assessment – FBBC Malnutrition screening score. A score of 3 or more warrants referral to a dietitian.
- Skin assessment
- Pressure mapping of clients on support surfaces when clinically indicated.
- Kalamunda Hospital does not have a pressure mapper. Where clinically indicated the Kalamunda Hospital Occupational Therapist can arrange to borrow a pressure mapper from AKG via the Senior Occupational Therapist – Disability Equipment and Home Modifications.
- Documentation of assessment should include any pressure mapping reports in the patient's medical record.

Pressure mapping is clinically indicated for clients:

- Newly prescribed a wheelchair for continual indoor use.
- Current stage 3 or 4 pressure injury/or are clinically assessed as high risk and require reassessment of their support surface/s.
- All non-palliative clients being discharged with a high risk support surface for home use – including short term loans.
- Deteriorating or not healing pressure injuries.

- **Intervention**

- Intervention will be planned and implemented in conjunction with clients/carers and multidisciplinary team members as appropriate, based on clinical need.

- **Issue of support surfaces**

- Support surfaces issued or ordered via Occupational Therapy require Occupational Therapy assessment.
- The issuing therapist is responsible for providing instruction and training in issued support surface, correct and safe use to the patient/carer. Verbal, practical demonstration and written instructions will be provided as appropriate.
- A basic range of support surfaces for trial and issue, and foam for manufacture of support surfaces is kept in stock in Occupational Therapy. Some cushions are in stock for trial only.
- All foam cushions are issued permanently as they cannot be cleaned to infection control standards.

- Support surfaces issued for community use must be issued in alignment with the AKG provision of aids/equipment and home modifications procedure.
- Where clients are not eligible for provision via AKG the client will be given information regarding private purchase. Where clinically indicated support surfaces can be issued on a short-term three month loan to allow adequate time for self or alternative provider purchase.
- Some items are for short-term loan only and require review at the end of the loan period. Length of loan will be a clinical decision
- All support surfaces issued must be clearly documented using the AKG Support surface record form (Appendix 1) with sufficient detail for another person to replicate the item if required.
 - 'Off the shelf' items require item code or description of exact item including size and type.
 - Custom designed items purchased externally require a copy of instructions to the supplier.
 - Support surfaces fabricated by AKG require exact dimensions and densities of foam used.

To comply to the Therapeutic Goods Administration Act it is essential to record replicable details for all support surfaces manufactured in-house at AKG.

- **Inpatient support surface use**
- Patient's assigned ward owned support surfaces by nursing staff (except mattresses) will be reviewed by Occupational Therapy.
- Each Armadale ward stocks a basic range of reusable support surfaces for use by inpatients on the ward. Where possible cushions prescribed for use on wards will be from the ward stock or in the case of Canning and MAU from the HospEquip hire stock.
- Kalamunda has a small stock of cushions for ward use stored in the therapy store area.
- Where a cushion is prescribed for use on the ward the "Pressure Care Cushion Set Up" sheet will be attached next to the mobility chart above the patient's bed to inform staff members of correct use and set-up and written in the DMR.
- **Education as clinically indicated can include:**
- General pressure injury prevention and warning signs. The patient handout: AKG Guidelines for Preventing Pressure Injuries, should be given to all clients/carers identified at risk of pressure injuries. This can be printed from the [HUB](#).
- Maintaining correct posture and positioning
- Regular repositioning, and movement,
- Transfer methods,
- Correct set up of equipment/household furniture to maintain best pressure distribution and prevent shear – including issue of equipment as appropriate and as per Provision of Aids/Equipment and Home Modifications Procedure.
- Correct use and care for any support surfaces issued.
- Referrals will be made to other health professionals as appropriate to clinical needs.

- **Inpatient discharge planning and follow-up**
 - Clients with short term loans – generally maximum three months; or hire of support surfaces on the ward should be reassessed post discharge.
 - Clients with current pressure injuries generally require ongoing review in the Community as per guidelines below.
 - Clients issued support surfaces for long term use will require replacement/review of these items. Foam cushions require review at least annually.
- **Clients discharged to residential facilities**
 - Foam support surfaces used by clients are issued for long term use.
 - Other support surfaces need to be provided by the residential facility.
 - Support surfaces can be issued on short term loan to allow the residential facility time to acquire the appropriate support surface.
 - Ongoing pressure care review and care is the responsibility of the residential facility.
 - Transfer document will be sent to the residential facility detailing ongoing pressure care review needs, recommended support surface, details of short term loans, recommended replacement date for issued foam support surfaces.
 - The Letter to Facilities re support surfaces should be used for this purpose (Appendix 2).
- **Clients discharged home - ineligible for outpatient/community services**
 - Provide information on support surface issued to the client including recommended review/replacement time – direct to client/carer and/or service provider as indicated.
 - Where ongoing pressure injury review is recommended information on accessing private services will be provided.
 - For clients accessing Community Aged Care Packages or NDIS information on ongoing care will be given to the provider.
- **Clients eligible for AKG Occupational Therapy Outpatient Care**
 - Clients can be referred for:
 - Ongoing review/replacement of long term loan supports surfaces.
 - Review of short term loan support surfaces.
 - Long term issue of a support surface via CAEP
 - Review of a current pressure injury as an outpatient.
 - Palliative clients and clients assessed as requiring short-term support use only do not require outpatient follow-up.
 - Referrals require an eReferral to OT Disability equipment and home Modifications with a copy of the support surface record and pressure mapping completed. The support surface record needs to be saved in W: Drive Pressure Care Clinic
- **Clients eligible for outpatient care by another health service**
 - Referral will be made to the appropriate catchment area service via e Referrals
 - For some services clients will also require referral via their GP. In these circumstances' clients will be advised to obtain the appropriate referral.

- **Occupational Therapy outpatient pressure care clinic follow-up**
- Clients with pressure injuries on initial assessment will be added to the seating clinic database for review at least annually until the injury is healed.
- Clients referred from the wards with short term loans will be evaluated post discharge for pressure injury risk as risk levels may have changed. Review may first be conducted by telephone with review in the clinic as indicated.
- Eligible clients issued long term loan support surfaces will have their support surfaces reviewed as clinically indicated.
 - Foam surfaces require review/replacement at least annually.
 - Review maybe by telephone initially - where there are no new changes/reports of discomfort, the Support Surface Record will be used to replicate the item which will be delivered to the client.
 - Review in the seating assessment clinic will occur as clinically indicated.
- For active support surfaces and shower commode cushions (as part of the shower commode service) an annual service will be completed in the client's home.
- All eligible clients will receive information on how to contact the Occupational Therapy department for repairs or replacement of support surfaces.
- Where clients are ineligible for follow-up, they or their carer or service provider as appropriate will be provided with details of recommended follow-up including the support surface issued and the review/replacement recommendation.

4. Supporting information

The following documents support the implementation of this procedure:

- [Allied Health Provision of Assistive Technology and Home Modifications Procedure AKG-PRO-0028-17](#)
- [Pressure Injury Prevention and Management Policy AKG-POL-0375-17](#)

5. Compliance, monitoring and evaluation

All Occupational Therapy staff require completion of Pressure Care Competency training which includes assessment of knowledge of this procedure. An annual audit will be conducted to check this has been completed.

6. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- **Standard 2 –Partnering with Consumers**

Integrating clinical governance

- 2.01 Clinicians use the safety and quality systems from the Clinical Governance Standard when:
 - a. Implementing policies and procedures for partnering with consumers
 - b. Managing risks associated with partnering with consumers
 - c. Identifying training requirements for partnering with consumers Applying quality improvement systems

- 2.02 The health service organisation applies the quality improvement system from the Clinical Governance Standard when:
 - Monitoring processes for partnering with consumers
 - b. Implementing strategies to improve processes for partnering with consumers
 - c. Reporting on partnering with consumers

- **Standard 3 – Preventing and Controlling Infections Standard**

Partnering with consumers

3.04 Clinicians use organisational processes consistent with the Partnering with Consumers Standard when assessing risks and preventing and managing infections, and implementing the antimicrobial stewardship program to:

- a. Actively involve patients in their own care
- b. Meet the patient's information needs
- c. Share decision-making

3:14 Clean and Safe Environment - The health service organisation has processes to evaluate and respond to infection risks for:

- a. New and existing equipment, devices and products used in the organisation
- b. Clinical and non-clinical areas, and workplace amenity areas
- c. Maintenance, repair and upgrade of buildings, equipment, furnishings and fittings
- d. Handling, transporting and storing linen
- e. Novel infections, and risks identified as part of a public health response or pandemic planning

- **Standard 5 – Comprehensive Care**

Integrating clinical governance

5.01 Clinicians use the safety and quality systems from the Clinical Governance Standard when:

- a. Implementing policies and procedures for comprehensive care
- b. Managing risks associated with comprehensive care
- c. Identifying training requirements to deliver comprehensive care Applying quality improvement systems

5.02 The health service organisation applies the quality improvement system from the Clinical Governance Standard when:

- a. Monitoring the delivery of comprehensive care
- b. Implementing strategies to improve the outcomes from comprehensive care and associated processes
- c. Reporting on delivery of comprehensive care Partnering with consumers

5.03 Clinicians use organisational processes from the Partnering with Consumers Standard when providing comprehensive care to:

- a. Actively involve patients in their own care
- b. Meet the patient's information needs
- c. Share decision-making Designing systems to deliver comprehensive care

5.04 The health service organisation has systems for comprehensive care that:

- a. Support clinicians to develop, document and communicate comprehensive plans for patients' care and treatment

- b. Provide care to patients in the setting that best meets their clinical needs
- c. Ensure timely referral of patients with specialist healthcare needs to relevant services
- d. Identify, at all times, the clinician with overall accountability for a patient's care

Collaboration and teamwork

5.05 The health service organisation has processes to:

- a. Support multidisciplinary collaboration and teamwork
- b. Define the roles and responsibilities of each clinician working in a team

5.06 Clinicians work collaboratively to plan and deliver comprehensive care

Clinicians use organisational processes from the Partnering with Consumers Standard when providing comprehensive care to:

- a. Actively involve patients in their own care
- b. Meet the patient's information needs
- c. Share decision-making

Screening of risk

5.10 Clinicians use relevant screening processes:

- a. On presentation, during clinical examination and history taking, and when required during care
- b. To identify cognitive, behavioural, mental and physical conditions, issues, and risks of harm
- c. To identify social and other circumstances that may compound these risks Clinical assessment

5.11 Clinicians comprehensively assess the conditions and risks identified through the screening process Developing the comprehensive care plan

5.12 Clinicians document the findings of the screening and clinical assessment processes, including any relevant alerts, in the healthcare record

5.13 Clinicians use processes for shared decision making to develop and document a comprehensive and individualised plan that:

- a. Addresses the significance and complexity of the patient's health issues and risks of harm
- b. Identifies agreed goals and actions for the patient's treatment and care
- c. Identifies the support people a patient wants involved in communications and decision-making about their care
- d. Commences discharge planning at the beginning of the episode of care
- e. Includes a plan for referral to follow-up services, if appropriate and available
- f. Is consistent with best practice and evidence.

Preventing and managing pressure injuries

5.21 The health service organisation providing services to patients at risk of pressure injuries has systems for pressure injury prevention and wound management that are consistent with best-practice guidelines

5.22 Clinicians providing care to patients at risk of developing, or with, a pressure injury conduct comprehensive skin inspections in accordance with best-practice time frames and frequency

5.23 The health service organisation providing services to patients at risk of pressure injuries ensures that:

- a. Patients, carers and families are provided with information about preventing pressure injuries
- b. Equipment, devices and products are used in line with best-practice.

- **Standard 6– Communicating for Safety**

Correct Identification and procedure matching.

6.05 The health service organisation:

- a. Defines approved identifiers for patients according to best-practice guidelines
- b. Requires at least three approved identifiers on registration and admission; when care, medication, therapy and other services are provided; and when clinical handover, transfer or discharge documentation is generated

6.06 The health service organisation specifies the:

- a. Processes to correctly match patients to their care
- b. Information that should be documented about the process of correctly matching patients to their intended care

Clinical handover

6.07 The health service organisation, in collaboration with clinicians, defines the:

- a. Minimum information content to be communicated at clinical handover, based on best-practice guidelines
- b. Risks relevant to the service context and the particular needs of patients, carers and families
- c. Clinicians who are involved in the clinical handover

6.08 Clinicians use structured clinical handover processes that include:

- a. Preparing and scheduling clinical handover
- b. Having the relevant information at clinical handover
- c. Organising relevant clinicians and others to participate in clinical handover
- d. Being aware of the patient's goals and preferences
- e. Supporting patients, carers and families to be involved in clinical handover, in accordance with the wishes of the patient
- f. Ensuring that clinical handover results in the transfer of responsibility and accountability for care

Documentation of information

6.11 The health service organisation has processes to contemporaneously document information in the healthcare record, including:

- a. Critical information, alerts and risks
- b. Reassessment processes and outcomes
- c. Changes to the care plan

References

1. Australian Wound Management Association. (2012). *Pan Pacific Clinical Practice Guidelines for the Prevention and Management of Pressure Injury*. Perth Australia: Cambridge Media.
2. National Safety and Quality Health Services Standards – Published 2021 revised 2023.

Glossary

The following definitions are relevant to this policy.

Term	Definition
Pressure injury	A localised injury to the skin and/or underlying tissue usually over a bony prominence, as a result of pressure, or pressure in combination with shear. (Previously referred to as pressure ulcers or pressure sores)
Support surface	A surface on which the patient is placed to manage pressure load. Support surfaces are classified as reactive (constant low pressure) or active (alternating pressure). Support surfaces include mattresses and overlays, seat cushions and overlays – including shower commode cushions, back cushions, heel and elbow protectors.

7. Policy owner (relating to these procedures)

Director Allied Health

Enquiries relating to this procedure may be directed to:

Title: Senior Occupational Therapist
 Department: Occupational Therapy - Hospital Wide
 Email: kim.johnson@health.wa.gov.au

8. Review

This procedure will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
V3	24/04/2024	24/04/2027	

9. Authorisation

This procedure has been authorised and issued by the Director Allied Health.

Authorised by	Danielle Kilmurray – Director Allied Health and Ambulatory Care
Authorisation date	24/04/2024
Published date	30/04/2024

Appendix 1: Support surface record template



Government of Western Australia
Department of Health



Armadale Health Service

Occupational Therapy

Support Surface Record

Name: UMRN:

Diagnosis:

Purpose:

Previous Pressure Injury:

If Yes, Location: Date:

Braden Score: Date:

Length of time sitting per day:

Reason for Issue:

Support surface:

Provider:

Cover/s:

Dimensions:

Date of Initial Provision:

Date of Reviews:

Next Date for Review:

Pressure Mapping:

Diagram:

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Delivering a Healthy WA



Appendix 2: Letter to facilities – support surface



Government of Western Australia
East Metropolitan Health Service
Armadale Kalamunda Group

Facility

/17

To Whom It May Concern:

Re

DOB:

The above mentioned client was provided with the following Support Surface(s) to assist with pressure care whilst an inpatient or outpatient at Armadale Kelmscott Memorial Hospital:

- ☐ **Foam Cushion** for pressure care while less mobile during their admission. This is the client's property and is suitable for their use only for approximately 12 months. After this time the cushion should be thrown away and the client assessed by the facilities Occupational Therapist for their future pressure needs.
- ☐ **Specialised Pressure Cushion** – this has been issued to the above mentioned client for their use only for 3 months. During this time the facilities Occupational Therapist can assess the client for ongoing needs and arrange for the facility to hire or purchase the support surface required for ongoing pressure care. The specialised cushion will then be collected by Armadale Health Service.
- ☐ **Specialised Pressure Relieving Mattress** called _____
and is still deemed necessary for the above clients pressure care needs. A list of suppliers is enclosed where this can be hired or purchased.

Thank you for your assistance in attending to the above client's ongoing pressure care needs.

Yours sincerely,

Occupational Therapist

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